Nexa Climate and Health Initiative 2026

FCHIP Climate Health Early Action

Climate-informed early warning that drives timely health action in Uganda

Your health, our mission.

Proof-of-Concept Fluxx answer bank | Uganda | Confirm orange fields before submit

Submission readiness gate

The strongest fit is Proof of Concept. Do not apply as Transition to Scale without credible, quantitative climate-health results. Do not submit until every orange field has been verified by the applicant.

Critical item	Current position
Applicant legal name and entity type	[CONFIRM BEFORE SUBMISSION]
Incorporated, active, and in good standing in Uganda	[CONFIRM BEFORE SUBMISSION]
Project Lead identity, role, email, and one-application confirmation	[CONFIRM BEFORE SUBMISSION]
Requested amount and 18/21/24-month duration	Amount draft USD 180,000; duration still [CONFIRM BEFORE SUBMISSION]
Collaborators and written roles	[CONFIRM BEFORE SUBMISSION]
Climate/weather data source and permission or licence	[CONFIRM BEFORE SUBMISSION]
District, facility, community, ethics, and data approvals	[CONFIRM BEFORE SUBMISSION]
Baseline values, sample, targets, and budget	Budget draft total USD 180,000; baselines/targets still [CONFIRM BEFORE SUBMISSION]
Prior GCC applications/awards and third-party IP	[CONFIRM BEFORE SUBMISSION]

Innovation Screen win checklist

Typically over 80% of applications are declined at the Innovation Screen, which reviews only the seven Overview answers. Paste Overview text only after this checklist is true.

- Area of focus named: climate-informed early warning and monitoring systems.
- Hazards in scope: changing mosquito ecology (malaria) and extreme heat; air quality out unless data and workflow are confirmed.
- Priority outcomes named: malaria; pregnancy hypertension / heat stress / GDM where captured; CVD and diabetes access.
- Climate-integrated framing: design, function, and deployment all shaped by climate factors.
- End-to-end loop: climate + health signals -> explainable trigger -> named actor -> service action -> follow-up.
- Health access/timeliness is the success bar — not alert accuracy alone; include intermediary system-response outcomes.
- Climate adaptiveness: compare performance across rainfall, heat, and service-pressure periods.
- Community connection at least linked/partnered from verified evidence; CHW/VHT co-design visible.
- PoC stage honest: working FairBanks field base; climate-health impact still to be proven — no TTS claims.
- Every CONFIRM BEFORE SUBMISSION field replaced before Fluxx submit.

1. Official call brief

Item	Verified information

Call	Nexa - Climate and Health Innovation: Africa, Latin America and the Caribbean
Co-leads	Grand Challenges Canada and the Science for Africa Foundation
Deadline	22 July 2026, 2:00 p.m. ET / 6:00 p.m. UTC
Submission	Fluxx only; complete application in English or French
Recommended stage	Proof of Concept; current evidence does not support Transition to Scale
Amount	Draft ask USD 180,000 (90% of PoC maximum USD 200,000)
Duration	18, 21, or 24 months
Applicant	Incorporated or equivalent in Africa or Brazil; Uganda is eligible
Implementation	Uganda is an eligible Proof-of-Concept implementation country
Community threshold	At least community linked
Climate bar	Must be climate-integrated: climate shapes design, function, and deployment
Innovation Screen	Only seven Overview answers reviewed first; typically over 80% declined there

1.1 Why this write-up can win

- Direct fit with climate-informed early warning and monitoring systems.
- Climate-integrated under Nexa's framework: climate shapes design, function, and deployment.
- End-to-end loop from climate and health signals to named service-delivery actions.
- Priority outcomes mapped: malaria; heat-sensitive pregnancy; CVD and diabetes access.
- Live FairBanks medical centre, Community Reach cascade, and CHW/VHT links for real-world PoC testing.
- Uganda is eligible; target groups match Nexa's climate-vulnerable priorities.
- Honest PoC ask: prove climate-responsive access and adaptiveness — not decorative weather data.

1.2 Relevant hazards and outcomes

Nexa scope	Proposed focus
Changing mosquito ecology	Malaria risk, prevention, testing, referral, and supply readiness
Extreme heat	Pregnancy hypertension/heat stress, cardiovascular disease, and diabetes care
Poor air quality	Out of initial scope unless reliable local data and respiratory workflow are confirmed
Priority groups	Pregnant women, children, older people, underserved urban communities, climate-sensitive NCDs

2. Proposed investment details

Portal field	Draft / action
Applicant organisation	[CONFIRM BEFORE SUBMISSION]
Project Lead	[CONFIRM BEFORE SUBMISSION] - formally affiliated, key team member, one application only

Project title (100 characters)	FCHIP Climate Health Early Action
Amount requested	USD 180,000 — 90% of PoC maximum USD 200,000
Duration	[CONFIRM BEFORE SUBMISSION] - select 18, 21, or 24 months
Collaborators	[CONFIRM BEFORE SUBMISSION] - list role and legal name
Country of incorporation	Uganda - confirm against incorporation documents
Legal entity type	[CONFIRM BEFORE SUBMISSION]
Country of implementation	Uganda
Community connection	[CONFIRM BEFORE SUBMISSION] - likely community partnered/linked; choose from evidence
Area of focus	Climate-informed early warning and monitoring systems
Language	English

3. Innovation Overview - exact character-limited answer bank

Only paste verified text. Character counts below include spaces and exclude the question label. Keep a small buffer because Fluxx may count formatting differently.

1. Specific climate-driven health gap [1,500]

In Kampala peri-urban communities served by FairBanks Community Reach — including Bukoto, Kyebando, Kisaasi, Kamwokya, and Kikaaya — climate change is shifting when and where health risk rises. Changing rainfall, temperature, and humidity alter mosquito breeding and malaria transmission, while extreme heat raises risk for pregnant women and people living with cardiovascular disease or diabetes. Local actors already collect pieces of the picture: CHW and VHT household reports, outreach screening, facility visits, medicine use, and weather information. Clinical records often stay locked inside existing EMR or hospital management systems, so community, climate, and facility signals rarely fuse in real time. The gap is not another stand-alone forecast. It is the missing end-to-end bridge from climate-driven risk signals to timely health service action — named owners, protocols, referral, follow-up, and supply readiness. Without that bridge, households face delayed prevention, testing, and continuity of care, and local systems cannot adapt services during rainfall or heat stress periods. FairBanks needs a practical climate-informed early warning and monitoring workflow that builds local resilience through earlier, targeted action — not a dashboard that stops at an alert.

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2. Proposed innovation [2,000]

FCHIP Climate Health Early Action is a climate-integrated FairBanks platform component on the Community Reach Data and Feedback loop. It directly addresses Nexa's climate-informed early warning and monitoring focus by integrating approved rainfall, temperature, humidity, and seasonal data with structured CHW/VHT reports, outreach screening, facility trends, medicine-use signals, and secure authenticated data APIs to existing EMR/HMS systems so clinical feeds can enter in real time without replacing software facilities already use. An offline-capable mobile workflow supports frontline capture and review. Explainable rules first score time-and-place malaria risk under changing mosquito ecology and heat-sensitive risk for pregnancy hypertension/heat stress, cardiovascular disease, and diabetes. A GIS action board shows risk, data quality, and agreed triggers. Each trigger links to a named service protocol: targeted messages, household follow-up, malaria testing and referral, outreach scheduling, clinician review, or medicine and supply preparation. This Proof of Concept will test whether the full loop — sense, interpret, act, learn — is feasible, accepted, timely, and able to improve access to climate-responsive care. Models will not diagnose or replace clinical judgement.

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3. Population design and access [1,500]

The pilot focuses on climate-vulnerable households in FairBanks Community Reach catchments in and around Kampala, including underserved peri-urban and informal settlement settings. Priority groups match Nexa's focus: pregnant women; children; older people; and people living with cardiovascular disease or diabetes — groups more exposed to malaria risk, heat stress, disrupted care, and financial barriers. CHWs and VHTs will help identify needs, test language and workflows, explain consent, collect only necessary data, and connect households to prevention, testing, referral, and follow-up. Community members will join design sessions, usability tests, feedback meetings, and review of what actions followed alerts, with gender-responsive and disability-aware engagement. Access will not depend on owning a smartphone: frontline workers will use offline-capable tools and existing outreach channels so care remains reachable during heat, heavy rain, or poor connectivity. Exact participation targets, sites, safeguarding steps, and compensation: [CONFIRM BEFORE SUBMISSION].

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4. Innovation in the target setting [2,000]

Current approaches often keep weather information, community reports, and clinical records in separate systems — including EMR/HMS platforms that do not easily share data with community tools. Many early warning products stop at forecasting and leave local teams without a named action pathway. FCHIP is designed as an end-to-end climate-informed early warning and monitoring system in Nexa's sense: monitoring and prediction linked to communication, preparedness, and timely health service delivery action. The core innovation is the climate-triggered action loop — climate plus community and facility signals, explainable trigger, named actor, protocol, referral, and follow-up result — not a weather layer bolted onto a health register. It combines technological innovation (offline capture, secure EMR/HMS data APIs, interpretable analytics, GIS), social innovation (CHW/VHT co-design and community feedback), and a delivery model rooted in a functioning medical centre and Community Reach programmes. It fits lower-resource and climate-stressed settings because it starts with simple validated rules, works offline, keeps clinicians responsible for care decisions, and does not require replacing existing EMR/HMS software. The pilot will compare timeliness, completion, acceptability, and cost with current manual planning before any machine learning is added.

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5. Potential to improve priority outcomes [2,000]

This Proof of Concept aims to improve health access — especially timeliness — for Nexa priority outcomes, not only alert accuracy. Primary pathways: (1) malaria — rainfall and community symptom signals trigger earlier prevention messages, testing, outreach, referral, and supply readiness; (2) pregnancy-related outcomes — heat thresholds trigger follow-up for hypertension, gestational diabetes risk where captured, and heat stress counselling; (3) climate-sensitive NCDs — heat triggers continuity checks and clinician review for cardiovascular disease and diabetes. We will measure whether more at-risk people receive a documented health service action after a climate-informed alert; whether median time from trigger to action falls; and whether referral and follow-up completion improve. Intermediary outcomes include local actors' capacity to interpret climate-health signals, ability to adapt outreach and surge during hazard periods, and continuity of essential services under rainfall or heat stress. Climate adaptiveness will be assessed by comparing performance across hazard and non-hazard periods. False alerts, missed events, equity of reach, and safety events will be reported. Baseline values and numeric targets: [CONFIRM BEFORE SUBMISSION]. Forecast accuracy alone will not count as success.

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6. Climate influence on design and deployment [1,500]

Under Nexa's climate-innovation framework, FCHIP Climate Health Early Action is a climate-integrated innovation: it began as community health intelligence and is being fundamentally re-engineered so climate factors shape the core design, functioning, and deployment. Design: rainfall, temperature, humidity, seasonality, GIS risk geography, and locally agreed hazard thresholds become first-class inputs, not optional add-ons. Function: explainable malaria and heat-risk rules, alert timing, and service protocols respond to climate conditions and data quality. Deployment: outreach, staffing, follow-up intensity, and supply preparation are scheduled by risk period and place, including during heat, heavy rainfall, and intermittent connectivity. The pilot will test climate integration in a real FairBanks catchment and document whether performance and service response change across climate-risk periods. Exact climate data provider, resolution, history, update frequency, licences, and thresholds: [CONFIRM BEFORE SUBMISSION].

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7. Team and community connection [1,500]

FairBanks has an active local presence through its medical centre and Community Reach cascade in Kampala-area communities including Bukoto, Kyebando, Kisaasi, Kamwokya, and Kikaaya. The cascade links community members, CHWs/VHTs, outreach programmes, clinical care, research and skills, and economic empowerment including CHIS where relevant — giving a practical route for co-design, referral, and follow-up. Key local roles include frontline CHW/VHT engagement, clinical review, outreach coordination, and community feedback. The application must still name the Project Lead, clinical and technical leads, CHW/VHT representatives, MEL and data-protection leads, and confirmed collaborators, with training, lived experience, location, role, and prior partnership outcomes. Select the community connection category only from verified evidence; do not claim community-owned or community-led status until leadership and residence evidence confirms it. Names and evidence: [CONFIRM BEFORE SUBMISSION].

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4. Full Proof-of-Concept application

These fields are reviewed after the Innovation Screen. They are application-ready structures, but facts and numbers marked for confirmation must be supplied before submission.

1. Execution plan [3,000]

Months 1-3 — Govern and co-design: confirm governance, communities, users, climate data source and licence, baseline indicators, malaria and heat response protocols, ethics and safeguarding; run design sessions with CHWs/VHTs, clinicians, and community representatives. Months 4-7 — Build: offline forms, secure EMR/HMS data APIs, data pipeline, explainable climate-health rules, GIS action board, audit trail, and role-based access. Months 8-10 — Test: train users; run a small usability and data-quality pilot; fix workflow friction before scale-up. Months 11-18 — Field pilot: operate malaria and heat-sensitive care workflows across selected sites and seasons, with monthly safety, equity, and learning reviews; refine thresholds using observed signals and service capacity. Months 19-21 — Analyse: quantitative and qualitative results, cost and sustainability scenarios, and climate-adaptiveness comparisons across hazard periods. Months 22-24 — Decide: validate findings with communities and partners, publish a responsible evidence package, and make a clear scale/no-scale decision. Learning and refinement are built into every stage. Exact sites, sample, milestones, owners, and duration (18/21/24 months): [CONFIRM BEFORE SUBMISSION].

2. Community engagement [2,000]

Use a human-rights and gender-responsive approach so marginalized voices shape design, testing, evaluation, and iteration. Fairly support CHW/VHT and community representation in co-design, usability tests, interpretation workshops, and governance reviews. Hold separate listening sessions where needed for pregnant women, older people, people with disabilities, and low-income or informal-settlement residents. Test language, consent, alert burden, referral barriers, preferred communication channels, and disability access. Publish a feedback route and show what changed after community input. Track participation by sex, age, disability where appropriate, and location without collecting unnecessary personal data. Safeguarding, complaints handling, and informed consent will be active throughout. Community representatives, compensation, safeguarding route, and engagement targets: [CONFIRM BEFORE SUBMISSION].

3. Risks and mitigation [2,000]

Key risks: weak or biased field data; false or missed alerts; alert fatigue; privacy harm; poor connectivity; low adoption; exclusion of women, older people, or people with disabilities; unsafe or incomplete referrals; partner delay; climate-data licensing limits; corruption or misuse of funds; and supply constraints after an alert. Mitigation: data minimisation and consent; role-based access, encryption, and audit logs; clinician oversight and conservative thresholds; offline workflows; named escalation and pause criteria; safeguarding and complaints handling; anti-fraud and segregated duties; partner agreements; and pre-agreed service capacity before triggering outreach. EMR/HMS APIs will use authentication, consent, and least-privilege scopes. Applicable Uganda approvals, ethics review, incident response, insurance, and organisational policies: [CONFIRM BEFORE SUBMISSION].

4. Expected impact [2,000]

Over the investment period we expect: improved local capacity to interpret climate-health signals; faster documented action after malaria and heat triggers; more at-risk people reached with prevention, testing, referral, or continuity-of-care support; improved referral completion; and better preparation of outreach and supplies during rainfall and heat periods. Reach and outcomes will be reported by priority group and location. Early signals of health-access impact are the PoC focus; we will not promise reductions in illness or hospitalisation without a powered evaluation design. Key assumptions: climate and health data arrive on time; service owners can act after alerts; communities accept the workflow; and ethics approvals are secured. Verified baseline, sample size, effect assumptions, targets, and attribution approach: [CONFIRM BEFORE SUBMISSION].

5. Proof of Concept and objectives [2,000]

Proof of Concept to establish: an end-to-end climate-informed early warning and monitoring workflow is technically feasible, acceptable, safe, affordable enough to continue, and able to improve timely access to named health service actions for malaria and heat-sensitive pregnancy/NCD care in the FairBanks catchment. Proposed measurable objectives: (1) achieve agreed completeness and timeliness for core climate and health data; (2) validate malaria and heat-risk triggers against observed local data with transparent false-positive/negative reporting; (3) reduce median time from trigger to documented service action versus baseline; (4) improve referral and follow-up completion for alerted cases; (5) reach priority groups equitably without serious safety or privacy events; and (6) produce a costed continuation decision from FairBanks and confirmed partners. Numeric targets and success thresholds: [CONFIRM BEFORE SUBMISSION].

6. Monitoring and evaluation [2,500]

Use a prospective mixed-methods pilot with a pre-defined theory of change linking climate signals to intermediary system response and priority health-access outcomes. Compare baseline and implementation periods and, if feasible, matched workflows or sites. Measure data quality; alert performance; response time; documented service actions; referrals; continuity of care; reach and equity; acceptability; adoption; cost per person reached; and safety events. Assess climate adaptiveness by comparing results across rainfall, heat, and service-pressure periods. Sources: system logs, CHW forms, facility records including approved EMR/HMS feeds, referral registers, climate data, surveys, interviews, and focus groups. Use confidence intervals and transparent missing-data analysis; report false positives and negatives. Limitations may include small sample, seasonal confounding, and incomplete comparator data — these will be stated openly. Independent statistician or research partner, final design, power/sample calculation, tools, approvals, and baseline: [CONFIRM BEFORE SUBMISSION].

7. Sustainability [2,000]

At this early stage, sustainability means designing for real operating cost and local ownership, not assuming grant-free scale. Build on existing FairBanks outreach and clinical workflows; use low-cost offline tools; train local users; document protocols; and test willingness and ability to continue after the grant. Future routes may include clinic subscriptions, programme implementation contracts, district or NGO deployments, and approved integrations including secure EMR/HMS data APIs. Barriers include climate-data cost, maintenance, workforce time, public-system fit, and reliable service capacity after alerts. The pilot will produce total-cost-of-ownership and financing scenarios and a clear scale/no-scale decision. Confirmed buyers, government pathway, partner commitments, and pricing evidence: [CONFIRM BEFORE SUBMISSION].

8. Project team [1,500]

FairBanks contributes local primary-care delivery, Community Reach outreach, CHW/VHT links, and community follow-up — the validation environment for this Proof of Concept. The project requires named leadership across clinical care, climate or epidemiology, product engineering, data protection, MEL, community engagement, finance, safeguarding, and partnerships, with time commitments matched to the workplan. Collaborators should close any skill gaps (for example climate data science, independent MEL, or district liaison) under written roles. Names, qualifications, employment links, CV evidence, and collaborator commitments: [CONFIRM BEFORE SUBMISSION].

9. Prior partnerships [1,500]

Describe only verified work with community groups, health facilities, government, academic, private-sector, and development partners. For each partner state dates, role, activity, and result so reviewers can judge stakeholder engagement capacity. Repository material supports an active community-health ecosystem but does not verify every formal partnership or outcome. Evidence and approved wording: [CONFIRM BEFORE SUBMISSION].

10. Budget

We request USD 180,000 — 90% of the Proof-of-Concept maximum (USD 200,000). Direct costs total USD 163,640 and indirect costs USD 16,360 (10% of direct costs, within the Fluxx 10% cap). The draft allocation funds remuneration for the core team and CHW/VHT stipends; subcontracted climate, MEL, and engineering support; Uganda travel for co-design and supervision; goods for alert-linked response and outreach; offline field equipment; project administration; and small community/VHT sub-grants. Most activities and expenses will occur in Uganda. Confirm salary scales, quotations, partner sub-grant agreements, currency assumptions, and final category totals before Fluxx certification: [CONFIRM BEFORE SUBMISSION].

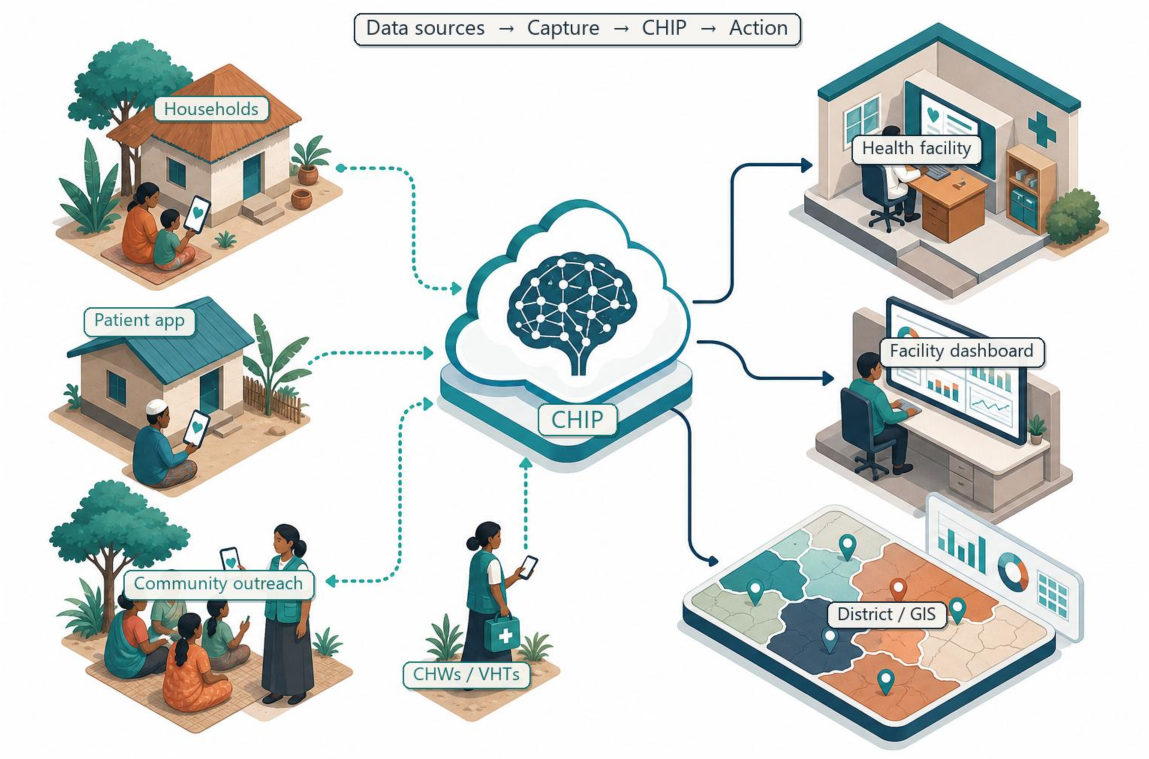
4.1 Budget workbook

Draft Fluxx allocation totaling USD 180,000 (90% of the USD 200,000 PoC ceiling). Confirm salary scales, quotations, and sub-grant partners before certification.

Fluxx category	Amount (USD)	Description / basis
Remuneration	74,000	Project Lead, clinical lead, product/data roles, MEL, community engagement, and CHW/VHT stipends across the investment period
Subcontractor fees	30,000	Climate/epidemiology support, independent MEL or statistician, and contracted engineering for offline capture, secure EMR/HMS APIs, and GIS action board
Travel costs	11,000	Community co-design and feedback visits, site supervision, partner and district meetings within Uganda
Goods and supplies	16,500	Pilot response supplies (e.g. malaria testing materials for alert follow-up), outreach materials, airtime/data, printing, and consumables

Equipment costs	13,500	Offline-capable phones/tablets for CHW/VHT capture, protective cases, and basic field accessories
Project administration costs	10,640	Finance coordination, banking fees, insurance allocation, and day-to-day project administration in Uganda
Sub-grants	8,000	Small community or VHT-linked implementation support for co-design, outreach, and feedback activities
Indirect costs	16,360	10% of direct costs (USD 163,640); within the 10% Fluxx cap
Total	180,000	90% of PoC maximum (USD 200,000); majority spend in Uganda

5. Product and action pathway



Illustrative FCHIP architecture; not evidence of a deployed climate-health product.

Layer	Proof-of-Concept role
Climate inputs	Rainfall, temperature, humidity, seasonality, and approved thresholds
Health inputs	CHW/VHT reports, outreach screening, facility trends, medicine use, and secure EMR/HMS clinical feeds
Secure EMR/HMS APIs	Authenticated, consent-aware APIs so existing facility systems share clinical data in real time
Responsible analytics	Explainable rules first; validation, monitoring, and clinician review
Action layer	GIS risk board, named response owner, protocol, referral, and follow-up
Learning loop	Response outcome, data quality, equity, safety, cost, and threshold refinement

5.1 Responsible AI and clinical safety

- Start with explainable thresholds and rules; add machine learning only after sufficient data and validation.
- Do not diagnose, prescribe, or automatically deny or delay care.
- Show uncertainty, data quality, and the reason for each alert.
- Require clinical review for care decisions and define escalation and pause criteria.
- Monitor bias, false alerts, missed events, workload, privacy, and unintended harm.

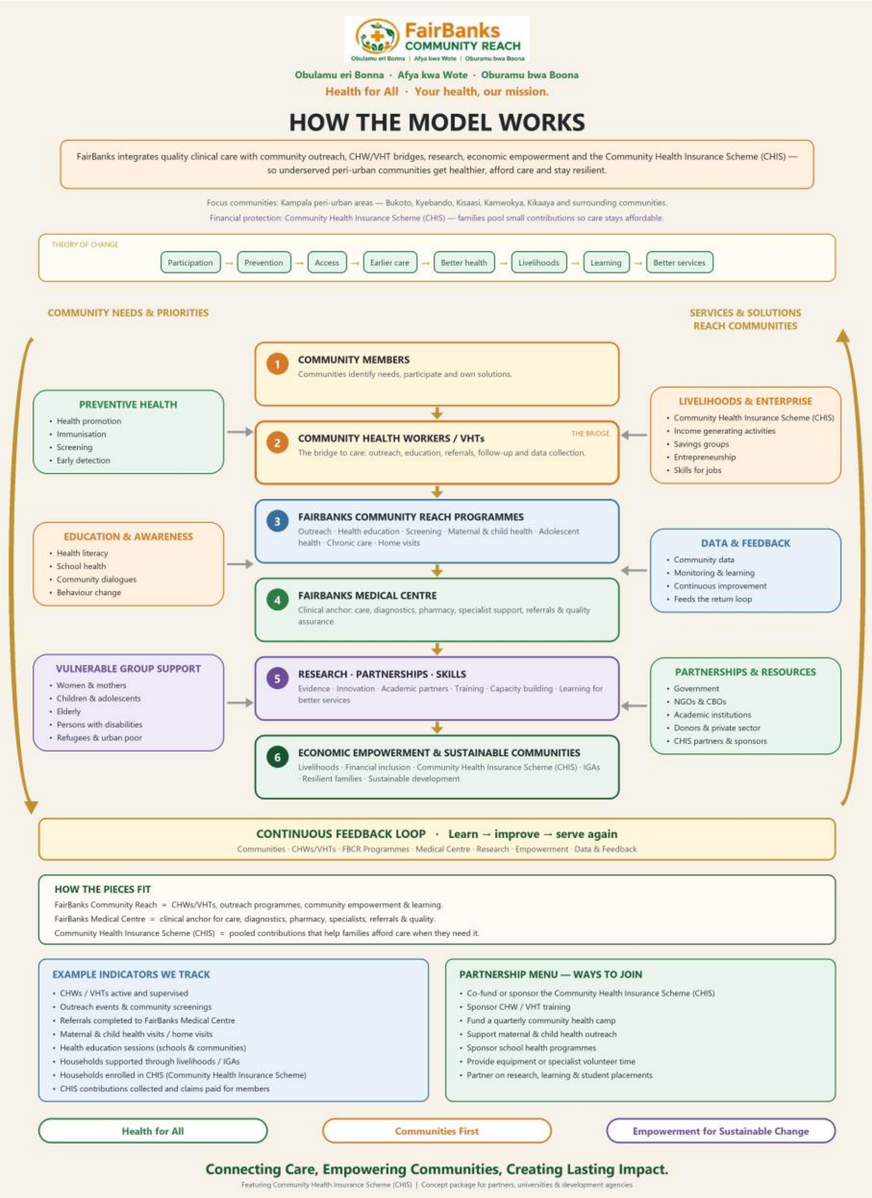
6. Monitoring, evaluation, and learning

Illustrative dashboard concept; measures and thresholds remain to be validated.

Domain	Measures
Data readiness	Completeness, timeliness, missingness, geographic coverage
Alert performance	Sensitivity, specificity, false alerts, missed events, lead time
Intermediary response	Local actor capacity to interpret signals; surge/outreach adaptation; service continuity
Priority outcomes	Timely prevention, testing, care access, continuity; malaria and heat-sensitive care
Climate adaptiveness	Performance compared across rainfall, heat, and service-pressure periods
Equity and safety	Reach by priority group, complaints, adverse events, privacy incidents
Feasibility	Adoption, acceptability, uptime, cost per person reached, staff burden

Nexa requires meaningful health access or health outcome evidence. Forecast accuracy alone is not enough.

7. Community Reach cascade and delivery



Canonical FairBanks Community Reach model. FCHIP sits on the Data and Feedback loop.

Cascade layer	Role in this Proof of Concept
1. Community members	Needs, participation, and household climate-health signals
2. CHWs / VHTs	Bridge: outreach, education, referral, offline data capture
3. Community Reach programmes	Screening, MCH, NCD, school/community education
4. FairBanks Medical Centre	Clinical review, diagnostics, pharmacy, referral QA; secure EMR/HMS APIs for real-time clinical ingest
5. Research · partnerships · skills	Evidence, climate expertise, training, ethics
6. Economic empowerment / CHIS	Affordable access and resilient households where data exists

- Community members identify needs and test usability, language, and fairness of workflows.
- CHWs and VHTs bridge households to prevention, screening, referral, and follow-up.
- Community Reach programmes coordinate outreach and health education.
- FairBanks Medical Centre provides clinical review, diagnostics, pharmacy, referral, and quality assurance.
- Research and partners support evidence, climate expertise, training, and ethical evaluation.
- Economic empowerment and CHIS support affordable access where enrolment data exists.
- Learning returns to communities and strengthens resilience, not only project reporting.

8. Risks and safeguards

Risk	Mitigation
Overclaiming climate fit	Make climate data and hazard-responsive action central, not decorative.
Weak evidence	Set baseline, comparison logic, sample, tools, and analysis before launch.
Unsafe automation	No diagnosis; clinician oversight, conservative thresholds, and pause rules.
Privacy and ethics	Minimise data, obtain consent/approval, control access, and log activity; EMR/HMS APIs use authentication and least-privilege scopes.
EMR/HMS friction	Expose stable documented APIs; start with FairBanks systems; do not require replacing existing EMR/HMS.
Exclusion	Offline CHW-mediated access; disability, gender, language, and poverty checks.
No capacity after alert	Pre-agree service protocols, owners, supplies, referral routes, and escalation.
Financial misuse	Segregated duties, procurement controls, declarations, audit trail, and reporting.

9. Additional portal information

Field	Action
Previous GCC application	[CONFIRM BEFORE SUBMISSION] - Yes/No and details
Previous GCC award	[CONFIRM BEFORE SUBMISSION] - Yes/No, grant IDs, and difference from prior work
Citations	Add recent, relevant climate-malaria, heat-health, CHW, and early-action evidence
Third-party IP	[CONFIRM BEFORE SUBMISSION] - include software, models, maps, climate and health datasets
Licences/access	[CONFIRM BEFORE SUBMISSION] - verify rights allow implementation and future scale
Certification	Authorised signatory must accept call terms and privacy notice
Confidentiality	Do not submit trade secrets or sensitive data that are unnecessary for review
AI disclosure	Applicant remains responsible for original, true, accurate, complete, and cited content

10. Final submission checklist

- Recheck the live call, deadline, and any revised documents.
- Register in Fluxx early and confirm the legal organisation record.
- Confirm Proof-of-Concept stage, amount, duration, and Uganda implementation.
- Replace every CONFIRM BEFORE SUBMISSION marker.

- Keep all seven Innovation Overview answers within character limits.
- Verify that climate factors shape design, function, and deployment.
- Link every risk signal to a named health service action and outcome.
- Complete the 24-month plan, MEL design, objectives, safeguards, and budget.
- Verify Project Lead, team, collaborators, community role, approvals, IP, and prior GCC history.
- Review for confidential information, plagiarism, unsupported claims, and citation quality.
- Submit in Fluxx before the deadline; save the confirmation.

11. Official sources

Source	URL
Official Nexa call page	https://www.grandchallenges.ca/rfp-nexa/
English Funding Opportunity	https://www.grandchallenges.ca/wp-content/uploads/2026/06/EN-Nexa-RFP-Jun-18-2026.pdf
Proof-of-Concept questions	https://www.grandchallenges.ca/wp-content/uploads/2026/06/Nexa-POC-application_for-webpage.pdf
Transition-to-Scale questions	https://www.grandchallenges.ca/wp-content/uploads/2026/06/Nexa-TTS-Stage-1-application_for-webpage.pdf
Fluxx application portal	https://gcc.fluxx.io
Nexa initiative site	https://www.nexaclimate.org

Source check date: 2026-07-20. Official Nexa documents and Fluxx always win.